National Cancer Control Programme (NCCP) 00689 · clinician draft

Atezolizumab + carboplatin + etoposide

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction Day 1 · carboplatin AUC 5

Active medicines: atezolizumab + carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk, interactions and patient factors.
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4	Carboplatin AUC≥4 dedicated NCIS V8 row Reconcile steroid indications and aprepitant interactions; one clear prescription.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS carboplatin row does not define an end day; confirm duration, renal suitability and contraindications.

Additional phase-specific support

- Dedicated carboplatin AUC≥4 row has no routine olanzapine; individualise if clinically indicated.
- Reconcile continued Day 1 prophylaxis with Days 2–3 etoposide cover and the two days after the last chemotherapy day. Do not duplicate antiemetics.
- No routine atezolizumab premedication unless previous reaction; after Grade 1–2 reaction consider antipyretic/antihistamine and close monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Induction Days 2–3 · etoposide and post-treatment cover

Active medicines: etoposide

Emetogenic risk: low

Medicine	Dose / route	When
Low-risk antiemetic reference	Ondansetron 8 mg orally once daily where selected	Before intravenous (IV) etoposide Days 2–3; review coverage through Day 5 NCIS low-risk row is a reference, not an additional automatic dose on top of Day 1 prophylaxis. Reconcile with ongoing aprepitant/dexamethasone and any 5-HT3 doses. Oral etoposide alternative requires its explicit local prescription.
Rescue antiemetic	Individual prescription	During treatment and later days NCIS low-risk row lists metoclopramide 10 mg orally three times daily as needed; account for any already scheduled doses and clinical suitability.

Additional phase-specific support

- NCIS multi-day principle: match prophylaxis to the drugs on each treatment day and continue appropriate cover for two days after.
- Etoposide Days 2–3 may be IV or an explicitly prescribed oral twice-daily alternative; no silent conversion.
- Issue a single reconciled day-by-day plan; do not start a fresh carboplatin high-risk course on each etoposide-only day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Maintenance · atezolizumab only

Active medicines: atezolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine chemotherapy antiemetic/steroid prophylaxis; provide an individual rescue plan if needed.
- Review the need for previous supportive prescriptions; do not stop required endocrine replacement or a therapeutic steroid taper without instruction.
- Continue alert card, cycle bloods, thyroid checks and prompt immune-symptom reporting.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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